

Specification Packaging Material

E-67052-04, LEAFLET MEDIXON TABLET MALAYSIA

Author(s):

29.10.2021, Helen Fransiska Manurung, Artwork Management Officer - BTR,

Reviewed by:

02.11.2021, Riski Amanda Tambunan, Supervisor Packaging Design,

Approved by:

02.11.2021, RA Officer 03, Regulatory Affairs Officer,

150 mm

100 mm

MEDIXON

Tablet

Composition:
Each tablet contains
Methylprednisolone 4 mg

Product Description:
White, round and flat bevelled edge tablet, diameter 7 mm, odorless. Side I marked DEXA, side II with breakline.

Pharmacodynamics:
Methylprednisolone is a glucocorticoid; prednisolone derivative which has the same activity to the main compound. Methylprednisolone has no sodium retention activity like the other glucocorticoids. Like other steroid hormones, corticosteroid act by controlling the rate of synthesis of proteins. However, their nearly instantaneous inhibition of ACTH release is probably an exception. Corticosteroids react with receptor proteins in the cytoplasm of sensitive cells in many tissues to form a steroid receptor complex. The complex undergoes modification, as noted by an increase in the sedimentation constant, and then moves into the nucleus, where it binds to chromatin and regulates transcription is enhanced, as manifest by increased amounts of specific mRNA. Nevertheless, glucocorticoids also decrease transcription of some genes, for example, the pro-opiomelanocortin gene that encodes ACTH.

Pharmacokinetics:
Absorption
Most glucocorticoids appear to be readily absorbed when administered orally as free alcohols, ketones, cypionates, or acetates.

Distribution
Most glucocorticoids are rapidly removed from the blood and distributed to muscles, liver, skin, intestines, and kidneys. Glucocorticoids vary in the extent to which they are bound to plasma proteins. Prednisolone (unlike other synthetic glucocorticoids such as betamethasone, dexamethasone, or triamcinolone) has a high affinity for transcortin and competes with cortisol for this binding protein. Because only an unbound drug is pharmacologically active, patients with low serum albumin concentrations may be more susceptible to effects of glucocorticoids than patients with normal serum albumin concentrations. Glucocorticoids cross the placenta and may be distributed into milk.

Elimination
Glucocorticoids having a ketone group at C-11 (e.g. cortisone, prednisone, and meprednisone) must be reduced (primarily in the liver) to their corresponding 11-hydroxy analogs (hydrocortisone, prednisolone and meprednisolone) in order to be pharmacologically active. Prednisone is rapidly converted to prednisolone, but much of cortisone is inactivated before it can be converted to hydrocortisone. Pharmacologically active glucocorticoids are metabolized in most tissues, but primarily in the liver, to biologically inactive compounds. The half-life of hydrocortisone may be prolonged in patients with hypothyroidism. Inactive metabolites are excreted by the kidneys, primarily as glucuronides and sulfates, but also as unconjugated products. Small amounts of unmetabolized drugs are also excreted in urine. Negligible amounts of most of the drugs are excreted in bile; enterohepatic circulation does not occur.

Indications:
Abnormality of adrenocortical function, allergic and skin inflammation, allergic and respiratory tract inflammation, hematologic disease, hypercalcemia associated with cancer and nervous system (acute exacerbations of multiple sclerosis).

Recommended Dose and Route of Administration:
Adults
Initial dosage of methylprednisolone may vary 4-48 mg daily.
Single dosage or multiple dosage depends on the condition being treated.
In a multiple sclerosis:
160 mg daily orally for 1 week, then 64 mg/two days for 1 month.

Children
Adrenocortical insufficiency:
Oral 0.117 mg per kg of body weight or 3.33 mg per m² of surface body wide in three divided doses.
Other indications:
Oral 0.417-1.67 mg per kg of body weight or 12.5 to 50 mg per m² of surface body wide in three or four divided doses.
Or as prescribed by physician.

The tablets are not meant to be cut into half.

Contraindications:
- Systemic fungal infections.
- Recent immunization and known hypersensitivity to the components.
- Long-term use of this drug is contraindicated to duodenal and peptic ulcer, osteoporosis, patients with a history of psychosis, herpes.

67052-04XX-YY
Last revised: dd/mm/yyyy

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Fiber direction

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Page 1

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150 mm

Warnings and Precautions:

- While on corticosteroid therapy, patients should not be vaccinated against smallpox or other immunization procedures especially on high doses, because of possible hazards of neurological complications.
- If corticosteroids are indicated in patients with latent tuberculosis or tuberculin reactivity, close observation is necessary as reactivation of the disease may occur.
- Corticosteroid effects may increase in patients with hypothyroidism or cirrhosis.
- Not recommended for patients with ocular herpes simplex because it may cause corneal perforation.
- Administered of this drug may mask some signs of infection and new infections may appear during their use.

Interactions with Other Medicaments:

- It is recommended to administer concomitantly with food to minimize gastrointestinal irritation.
- Administered concomitantly with other nonsteroidal anti-inflammatory drugs or antirheumatic drugs may cause gastrointestinal hemorrhage.
- If this drug is administered concomitantly with antidiabetic drugs, the dosage should be adjusted.
- Patients who undertake smallpox vaccination or other immunization procedures especially on high dose.
- Concomitant administration of prednisolone and ciclosporin may result in decreased plasma clearance of methylprednisolone, and plasma concentrations of ciclosporin may be increased during concomitant therapy with methylprednisolone.
- Seizures reportedly have occurred in adult and pediatric patients receiving high-dose methylprednisolone therapy concurrently with ciclosporin.
- Drugs such as barbiturates, phenytoin and rifampin which induce hepatic enzymes may increase glucocorticoid metabolism, and patients stabilized on glucocorticoid therapy may require dosage adjustments if such drugs are added to or withdrawn from their drug regime.
- Aspirin should be used cautiously in conjunction with glucocorticoids in patients with hypoprothrombinemia. Corticosteroids also increase risk of gastrointestinal bleeding and ulceration with aspirin.
- Ketoconazole increases serum methylprednisolone's adrenal suppressive effects.
- Troleandomycin inhibits the elimination of methylprednisolone.
- Response to anticoagulants may be altered by corticosteroids.

Pregnancy and Lactation:

Not recommended in pregnant women and nursing mother, except if clearly needed. Infants born to mothers who have received substantial doses of corticosteroid during pregnancy should be carefully monitored for symptoms of adrenal insufficiency.

Adverse Effects:

Adverse effects occur in a prolonged usage or in a large dose e.g.:
Fluid and electrolyte disturbances, muscle weakness, impaired wound healing, increased blood pressure, posterior subcapsular cataracts, suppression of growth in children, adrenal insufficiency, Cushing's syndrome, osteoporosis, and peptic ulcer.

Overdose and Treatment:

There is no clinical syndrome of acute overdosage. Methylprednisolone is dialyzable.

Presentation and Registration Number:

Box, 10 blisters x 10 tablets; MAL19990587AZ

Shelf Life:

4 years.

ON MEDICAL PRESCRIPTION ONLY.

STORE AT TEMPERATURES BELOW 30°C, IN A DRY PLACE. PROTECT FROM LIGHT.

Manufactured by
PT Dexa Medica
Jl. Jend. Bambang Utuyo No. 138
Palembang-Indonesia

Product Registration Holder
Pharmaforte (M) Sdn. Bhd.
2, Jalan PUJ 3/49, Sunway Damansara,
47810 Petaling Jaya,
Selangor-Malaysia

Date of revision: 19 October 2021

Fiber direction

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Black

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No.	Parameters	Specification
1.	Material	HVS (Lihat CoA / <i>See CoA</i>)
2.	Gramatur / <i>Grammature</i>	54 – 66 gsm
3.	Dimensi panjang / <i>Length dimension</i>	149 – 151 mm
4.	Dimensi lebar / <i>Width dimension</i>	99 – 101 mm
5.	Arah serat / <i>Fiber direction</i>	Horisontal / <i>Horizontal</i>
6.	Warna / <i>Colour</i>	Lihat proof print / <i>See proof print</i>
7.	Redaksional / <i>Redactional</i>	Lihat proof print / <i>See proof print</i>

HISTORY

Revision No.	Revision Date	Details	Reason
00	-	- Dokumen baru menggantikan SPM-E-67052-01 - Perubahan NIE Malaysia	CC001976
01	15-Jul-16	Penambahan keterangan tanggal revisi terakhir dan perubahan distributor	Email RA 15 Juli 2016
02	22-Jul-16	Penambahan tanggal revisi terakhir	Approval Distributor 21 Juli 2016
03	29-Jan-18	- Perubahan redaksional	TD Medixon 09.01.2018
		- Penghilangan ® setelah nama brand	Email Marketing OBU 22.03.2018
		- Perubahan dimensi	Mengikuti Drawing Leaflet Cartoning IMA
		- Penambahan parameter warna dan redaksional pada spesifikasi	MoM Packdev-TS 29.08.2017
04	07 Mei 2019	- Perubahan dokumen menjadi general use - Penambahan approval site	CC004303
05	22 Jun 2020	- Revisi item number menjadi E-67052-04 - Perubahan alamat Dexa Medica menjadi: Jl. Jend. Bambang Utoyo No. 138 Palembang - Indonesia	CMCOthRec062491/1
06	29 Okt 2021	- Perubahan product description: White, round and flat bevelled edge tablet, diameter 7 mm, odorless. Side I marked DEXA, side II with breakline - Penambahan statement "The tablets are not meant to be cut into half" pada section dosage	CMCOthRec079143/1